PEDIATRIC BRAIN SHEET

NICU · Peds · ED · Urgent Care Nursing Reference

v2.0 — Clinical Reference
Print: Letter · 1-2 Pages

PATIENT: Name _____ **DOB:** MM/DD/YYYY _____ **MR#:** _____ **AGE:** Y/M/D _____ **⚠ ALLERGY** List allergies _____

DATE: _____ **RN:** _____

1 WEIGHT & DOSAGE QUICK-CALC

WEIGHT

_____ **kg**

LB _____ OZ _____

DOSE CALCULATIONS

ML/KG	MCG/KG/MIN
_____ mL	_____ mcg
MG/KG	**UNITS/KG/HR**
_____ mg	_____ u

BSA (M²) _____ **EGFR** _____

2 PEDIATRIC GLASGOW COMA SCALE

Age < 2 Years — Use Pediatric Modifications		
Eye	4 = Spontaneous	☐
	3 = To voice	☐
	2 = To pain	☐
	1 = None	☐
Verbal	5 = Coos, babbles	☐
	4 = Irritable cry	☐
	3 = Cries to pain	☐
	2 = Moans	☐

3 FEBRILE NEONATE <30 DAYS

⚠ SEIZURE RISK · BABIES <30 DAYS
Fever >38°C (100.4°F) = FULL SEIZURE WORKUP

TEMP RANGE
38.0–38.5°C: _____
>38.5°C: _____

WORKUP
- ☐ CBC, BCx2
- ☐ UA, UCx2
- ☐ LP (if >21 days)
- ☐ CXR if indicated

ANTIBIOTICS REMINDER
Ampicillin 75–100 mg/kg
+ Cefotaxime 50 mg/kg
OR Gentamicin 2.5 mg/kg
ADD:
Acyclovir 20 mg/kg if HSV risk

WBC: _____ **ANC:** _____
BANDS: _____ **CRP:** _____

4 DEVELOPMENTAL MILESTONES

AGE	MOTOR	SOCIAL/COGNITIVE
Infant 0–12 mo	Head control 2-3mo, rolls 4mo, sits 6mo, crawls 8mo, walks 12mo	Social smile 2mo, stranger anxiety 6-8mo, waves bye 9mo, mama/dada 12mo
Toddler 1–3 yr	Runs 18mo, kicks ball 2yr, climbs stairs 2yr, jumps 3yr	Parallel play 2yr, uses spoon 2yr, toilet training 2-3yr, follows 2-step commands
Preschool 3–5 yr	Hops 3yr, rides tricycle 4yr, skips 5yr, draws circle 3yr, square 4yr	Plays cooperatively 4yr, knows first/last name 3yr, knows gender 4yr, counts 4yr
School-age 6–12 yr	Refines coordination, plays sports, writes fluidly 7yr	Concrete operations, understands rules, friendships important, 8-10hr sleep needed
Adolescent 12–18 yr	Pubertal growth spurt, adult body proportions, fine motor adult-like	Abstract thinking emerges 12+, identity formation, peer relationships primary, risk-taking

5 PEDIATRIC VITAL SIGNS BY AGE

AGE	HR (BPM)	RR (/MIN)	SBP (MMHG)	TEMP (°C)
Neonate 0-28d	100-160	**30-60**	60-90	**36.5-37.4**
Infant 1-12mo	80-150	**25-45**	70-100	**36.5-37.4**
Toddler 1-3yr	70-130	**20-35**	80-110	**36.5-37.4**
Preschool 3-5yr	60-120	**18-28**	85-115	**36.5-37.4**
School 6-12yr	55-100	**16-24**	90-120	**36.5-37.4**
Adolescent 12+	50-90	**14-20**	100-130	**36.5-37.4**

PEDIATRIC SBP FORMULA: Lower limit SBP = 70 + (2 × age in years) mmHg
MAP: DBP + 1/3(SBP - DBP) | **PULSE OX:** ≥94% (≥92% if neonate)

CURRENT HR: _____ **RR:** _____ **BP:** _____
T: _____ **SPO₂:** _____

6 COMMON MEDS QUICK REFERENCE

Acetaminophen (Tylenol)
10-15 mg/kg/dose PO/PR q4-6h
MAX: 75 mg/kg/day (children)
Neonates: 10-12 mg/kg/dose q6-8h

Amoxicillin 20-40 mg/kg/day ÷ 3
AUGMENTIN: 25-45 mg/kg/day ÷ 2
Azithromycin 10 mg/kg Day 1, then 5 mg/kg Days 2-5
Ceftriaxone 50-75 mg/kg/day ÷ 1-2

Ibuprofen 5-10 mg/kg/dose q6-8h
MAX: 40 mg/kg/day
Avoid if: <6mo, dehydration, renal dz

Ondansetron 0.1 mg/kg/dose (min 2mg)
Methylprednisolone 1-2 mg/kg/dose
Dexamethasone 0.6 mg/kg (croup)

▶ RESPIRATORY DISTRESS

CROUP (Laryngotracheobronchitis)
Dx: Barky cough, stridor, URI prodrome
Mild: Intermittent stridor at rest
Mod: Stridor + retractions, agitated
Severe: Tripod, silent child, cyanosis
Rx: Dexamethasone 0.6 mg/kg PO/IM
Nebulized EPI (1:1000) if severe
Keep calm → cold mist, HOB elevated

BRONCHIOLITIS (RSV)
Age: <2 years (peak 2-6mo)
S/S: Wheeze, cough, tachypnea, retractions
Mild: SpO_2 ≥92%, no/mild retractions
Mod: SpO_2 88-92%, moderate WOB
Severe: SpO_2 <88%, severe WOB, apnea
Rx: Suctioning, O_2, hydration
NO: Albuterol routine, steroids, antibiotics

ASTHMA SEVERITY
Mild: SpO_2 ≥92%, talks in sentences
Mod: SpO_2 88-92%, talks in phrases
Severe: SpO_2 <88%, can't complete sentences
Rx: MDI Albuterol q20min × 3, then q1-4h PRN
Ipratropium q20min × 3 (first hour)
Oral Steroids: Prednisolone 2 mg/kg
EF: Magnesium sulfate if refractory

▶ SEIZURE MANAGEMENT

STATUS EPILEPTICUS
≥5 min continuous seizure OR ≥2 in 24hr
0-5 min: ABCs, O_2, Glucose check
5-10 min: Lorazepam 0.1 mg/kg IV
OR Midazolam 0.2 mg/kg IN
10-20 min: Fosphenytoin 20 mg/kg
>20 min: Phenobarbital 20 mg/kg
>40 min: Anesthesia / ICU

SEIZURE SAFETY
- ☐ Time the seizure
- ☐ Position on side
- ☐ Clear area of hazards
- ☐ Do NOT restrain
- ☐ Do NOT put anything in mouth
- ☐ Check glucose
- ☐ O_2 via NC

FEBRILE SZN: Tx fever >38°C

BEFORE DISCHARGE: Neuro exam normal, eating/drinking, parents taught seizure safety

▶ DEHYDRATION ASSESSMENT

SIGN	MILD 3-5%	MOD 6-9%	SEVERE ≥10%
HR	N	↑	↑↑ / ↓
BP	N	N to ↓	↓↓
Mucosa	Moist	Dry	Parched
Tears	N	↓↓	Absent
Fontanelle	N	↓	Sunken
Skin	Snaps back	Slow	Very slow
Cap refill	<2 sec	2-3 sec	>3 sec
Urine	Dark	<1mL/kg/hr	Minimal

Replace: Mild=ORS 50mL/kg, Mod=ORS 100mL/kg, Severe=IV bolus 20mL/kg then D5 0.45%NS

▶ DIABETIC KETOACIDOSIS (DKA)

DIAGNOSIS
Glucose >200 + pH <7.3 + HCO_3 <18
Anion gap elevated, + ketones
Serum K may be NORMAL or HIGH

FLUIDS
0.9% NS 10-20 mL/kg bolus
Then D5 0.45% NS + 20 mEq/L KCl
Insulin drip 0.05-0.1 u/kg/hr
Goal: Dec BG 50-75 mg/dL/hr
Correct over 48-72hr **SLOWLY**

CEREBRAL EDEMA WARNING: HA, AMS, bradycardia, ↓LOC → Mannitol 0.5-1 g/kg, hold fluids, head up

NURSING NOTES
Assessment, interventions, family communication...